

DEMENTIA

• **Ensure a doctor confirms the diagnosis of dementia. Consider dementia in the patient who for at least 6 months has the following, which are getting worse:**

- Problems with memory: test this by asking patient to repeat 3 common words immediately and then again after 5 minutes.
- Disorientated to time (unsure what day/season it is) and place (unsure of shop closest to home or where the consultation is taking place).
- Difficulty with speech and language (unable to name parts of the body).
- Struggles with simple tasks, decision making and carrying out daily activities.
- Is less able to cope with social and work function.
- If patient has HIV, has difficulty with coordination.

Assess the patient with dementia with the help of the carer

Assess	When to assess	Note
Symptoms	Every visit	• If recent change in mood, energy/interest levels, sleep or appetite, consider depression and discuss/refer. • If suicidal thoughts or plans ➔ 83. • If sudden deterioration in behaviour ➔ 85. • If hallucinations (seeing or hearing things), delusions (unusual/bizarre beliefs), agitation or wandering, discuss/refer to mental health practitioner. • Manage other symptoms as on symptom pages.
Side effects	If on treatment	If abnormal movements or muscle restlessness, stop treatment and discuss/refer same day.
Vision/hearing problems	Every visit	Refer to optometry/audiology services for testing and proper devices.
Nutritional status	Every visit	Ask about food and fluid intake. If BMI < 18.5 arrange nutritional support. BMI = weight (kg) ÷ height (m) ÷ height (m).
Palliative care	Every visit	If any of: bed-bound, unable to walk and dress alone, incontinence, unable to talk meaningfully or do activities of daily living, also give palliative care ➔ 170.
BP	At diagnosis	If known hypertension ➔ 133. If not, check BP: if BP ≥ 140/90 ➔ 132.
CVD risk	At diagnosis, then depending on risk	Assess CVD risk ➔ 127.
HIV	At diagnosis or if status unknown	Test for HIV ➔ 110. If HIV positive, give routine care ➔ 111. If new HIV diagnosis with dementia, discuss with specialist.
Syphilis	At diagnosis	If positive, treat ➔ 53 and refer.
Thyroid function	At diagnosis	Check TSH. If abnormal, refer.
Glucose	At diagnosis	If known diabetes ➔ 130. If not known with diabetes, check glucose ➔ 17.

Health for All

➔ 112

Advise the patient with dementia and his/her carer

- Discuss what can be done to support the patient, carer/s and family. Identify local resources, social worker, counsellor, NGO ➔ 178. Refer to occupational therapy if available.
- Discuss with carer if respite or institutional care is needed. Advise the carer/s to:
 - Give regular orientation information (day, date, weather, time, names)
 - Use simple short sentences.
 - Remove clutter and potential hazards at home.
 - Stimulate memories and give current information with newspaper, radio, TV, photos.
 - Maintain a routine.
 - Maintain physical activity and plan recreational activities.

Treat the patient with dementia

- If HIV positive, ensure patient on ART ➔ 111, as HIV-associated dementia often responds well to ART.
- If aggression, wandering, night-time disturbance or psychotic symptoms or anxiety, discuss/refer. Avoid benzodiazepines (lorazepam, diazepam, midazolam) if > 65 years.

Review the patient with dementia every 6 months.